BASELINE → TREATMENT → FOLLOW-UP

# When checks happen

Relative milestones, not equal time intervals. The clinical team owns booking, result review and action on overdue tests. This is an educational map, not an automated appointment or result tracker.

| Check | Before starting | Early cycles | Later cycles / transition | Next treatment / completion | Follow-up |
|---|---|---|---|---|---|
| **Blood tests** | full blood count (FBC), kidney and liver profile | Review before chemotherapy cycles | Check again at any phase change | Follow active medicines and abnormalities | Follow unresolved abnormalities |
| **Echo / heart** | Baseline echo (ESC); assess risk | High cardiac risk: repeat after 2 anthracycline cycles | If continuing: repeat after 4 cycles at high risk | Review cardiac symptoms and total lifetime exposure | Echo within 12 months; high risk within 3 months |
| **electrocardiogram (ECG)** | Baseline ECG in the National Cancer Control Programme (NCCP) anthracycline source; repeat for symptoms or clinical risk. An ECG does not replace an echo. | Symptoms / risk-based repeat | Review if cardiac concern | ECG and echo answer different questions | Clinical indication |
| **Cancer imaging** | Record baseline extent as appropriate | Clinical response / symptoms review | If before surgery: plan response review | Arrange selected surgery / next treatment | Planned breast imaging; no routine computed tomography (CT)/ positron emission tomography (PET) |
| **Markers / bone health** | Tests selected for active medicines | No routine tumour-marker clearance | No routine blood tumour markers when well | dual-energy X-ray absorptiometry (bone density) (DEXA) if endocrine/ bone-risk indication | Follow the endocrine and bone-health plan |

The team arranges and reviews tests. This guide shows expected timing; it does not record completed appointments. NCCP 00252 v7 · official regimen · ESMO 2024 early breast cancer guideline · treatment and follow-up · ESC 2022 cardio-oncology guideline · cardiac surveillance and risk · NICE NG101 · bone health and follow-up · ESMO metastatic breast cancer guideline · 2026

THE WHOLE TREATMENT JOURNEY

# AC · every 3 weeks: treatment and checks

## Adjuvant breast cancer

Doxorubicin and cyclophosphamide on Day 1 every 21 days for four cycles. Any subsequent taxane follows a separately agreed regimen.

**Read from left to right.** Columns show stages, not equal lengths of time. This is a general plan: no dates or personal information are needed. Your team confirms which steps apply and arranges appointments.

**Treatment, tests and reviews across the course**

| Care / check | Before starting | Early cycles | Later cycles / transition | Next treatment / completion | Follow-up |
|---|---|---|---|---|---|
| **Cancer medicines** NCCP 00252 | Agree aim, indication and medicines | AC · every 3 weeks starts | Review response, toxicity and exposure | Complete planned course / next decision | Ongoing review / subsequent plan |
| **Surgery** EARLY | Adjuvant: surgery already completed | No automatic operation during cycles | Review response / surgical plan | Surgery after neoadjuvant therapy, if applicable | Pathology and recovery guide next steps |
| **Radiotherapy** EARLY | Assess indication | Usually after cytotoxic chemotherapy | Arrange sequential timing | Radiation team confirms start | Review effects / follow-up |
| **Blood tests** NCCP 00252 | FBC, kidney and liver profile | Review before chemotherapy cycles | Check again at any phase change | Follow active medicines and abnormalities | Follow unresolved abnormalities |
| **Echo / heart** NCCP 00252 · ESC | Baseline echo (ESC); assess risk | High cardiac risk: repeat after 2 anthracycline cycles | If continuing: repeat after 4 cycles at high risk | Review cardiac symptoms and total lifetime exposure | Echo within 12 months; high risk within 3 months |
| **ECG** NCCP 00252 | Baseline ECG in the NCCP anthracycline source; repeat for symptoms or clinical risk. An ECG does not replace an echo. | Symptoms / risk-based repeat | Review if cardiac concern | ECG and echo answer different questions | Clinical indication |
| **Cancer imaging** NCCP 00252 | Record baseline extent as appropriate | Clinical response / symptoms review | If before surgery: plan response review | Arrange selected surgery / next treatment | Planned breast imaging; no routine CT/PET |

| Care / check | Before starting | Early cycles | Later cycles / transition | Next treatment / completion | Follow-up |
|---|---|---|---|---|---|
| **Markers / bone health**<br>NCCP 00252 | Tests selected for active medicines | No routine tumour-marker clearance | No routine blood tumour markers when well | DEXA if endocrine/bone-risk indication | Follow the endocrine and bone-health plan |
| **Endocrine / bone**<br>BONE · EARLY | Confirm hormone receptors | Plan endocrine therapy when appropriate | Separate from this chemotherapy prescription | Start/continue per multidisciplinary team (MDT); DEXA when indicated | Risk-based bone review |

# What each check is for

## 1 Before first dose

Baseline

Baseline FBC, kidney and liver tests; repeat before each chemotherapy cycle.

**Ask / act:** Confirm tests and consent before prescribing.

Sources: NCCP 00252

## 2 Heart-monitoring plan

Baseline → cycles → after treatment

ESC supplementary guidance: baseline echo before anthracycline treatment. High/very-high cardiac risk: echo every two anthracycline cycles (for example before cycles 3 and 5 when continuing), and within 3 months after completion. All adults: echo within 12 months after completion. Other repeat tests depend on risk, cumulative exposure and symptoms.

**Ask / act:** Name the team responsible for booking and reviewing results.

Sources: NCCP 00252 · ESC

## 3 Treatment-phase review

Before each transition

Doxorubicin and cyclophosphamide on Day 1 every 21 days for four cycles. Any subsequent taxane follows a separately agreed regimen.

**Ask / act:** Confirm active medicines, support script and the next test checkpoint.

Sources: NCCP 00252

## 4 Response and follow-up

Planned review points

For early breast cancer, breast imaging and clinical review follow the agreed pathway. Routine CT/PET scans and blood tumour markers are not used to look for distant recurrence when there are no symptoms or clinical concerns.

**Ask / act:** Review symptoms and imaging together; do not change treatment on tumour markers alone.

Sources: NCCP 00252

## At the next review, ask

- Which treatment phase am I in, and what happens next?
- Which test is expected next, who arranges it, and who explains the result?
- Who should I contact if an expected appointment has not arrived?

This map does not show whether a test has been ordered, booked or completed. Do not wait for a scheduled test if you develop new symptoms.

Sources checked 2026-10-23 · Source reconciled; independent clinical, pharmacy and patient/PPI review pending · SACTCheck v0.84.17